DSIP (Delta Sleep Inducing Peptide)

Step-by-Step Reconstitution Process

- **Gather supplies: BAC water, alcohol swabs, insulin syringe, vial**
- Bring BAC water and peptide vial to room temperature before mixing
- **Removing plastic cap.** Carefully pop off the plastic cap, do not remove metal flange.
- **Clean both vials:** Wipe the rubber stoppers with alcohol swabs
- **Draw BAC water:** Pull your chosen amount into the syringe
- **Inject slowly:** Add the water to the peptide vial, aiming it at the glass wall—not directly onto the powder
- **Swirl gently:** Roll the vial between your palms. Never shake.
- **Wait:** Let it sit 5-10 minutes. Solution should be crystal clear.
- **Store:** Refrigerate immediately. Good for 28-30 days once reconstituted.

If it's cloudy or won't dissolve: Let it sit longer. Try gentle warming (under your arm or in lukewarm water). If still cloudy after 30 minutes, contact your source.

Practical Protocols

The following protocols are not derived from published dose finding studies. They represent clinical practice patterns and community consensus.

None

STANDARD PROTOCOL

Dose: 100 to 300 mcg per day

Frequency: Once daily

Timing: 30 to 60 minutes before bed

Administration: Subcutaneous injection

Duration: 5 to 10 consecutive nights, then evaluate

None

BY BODY WEIGHT

Under 150 lbs: 100 to 150 mcg nightly

150 to 200 lbs: 150 to 250 mcg nightly

Over 200 lbs: 200 to 300 mcg nightly

None

BY APPLICATION

General sleep optimization: 100 to 200 mcg nightly

Stress related sleep issues: 200 to 300 mcg nightly

GH peptide sleep disruption: 100 to 200 mcg nightly

Shift work sleep support: 200 to 300 mcg before target sleep

time

None

RECONSTITUTION (standard)

Vial size: 10 mg

Bac water: 2 mL

Concentration: 500 mcg per 10 units on insulin syringe

For 200 mcg: 4 units on insulin syringe

Cycling

There is no established cycling protocol from published research. Based on community experience and the theoretical concern about receptor adaptation, common approaches include using it for 5 to 10 consecutive nights then taking a break, or using it 3 to 5 nights per week rather than every night. If you find it stops working after prolonged daily use, take a 1 to 2 week break.

Timing

Administer 30 to 60 minutes before your target sleep time. Earlier timing (2 to 3 hours before bed) has been reported as less effective by some users. The onset is relatively quick at 10 to 30 minutes.

Common Questions

Is DSIP a sleeping pill? No. Sleeping pills like benzodiazepines and Z-drugs force sedation by overriding your brain's natural mechanisms. DSIP enhances your natural sleep architecture without forcing unconsciousness. It promotes slow wave sleep, which is the deep restorative phase, without suppressing REM.

Will DSIP help my chronic insomnia? Maybe modestly. The one controlled study on chronic insomnia found DSIP produced only weak improvements and concluded it was "not likely to be of major therapeutic benefit." An open study showed better results over 10 sessions with benefits

lasting months, but that lacked a placebo control. DSIP appears better suited for optimizing decent sleep than for treating severe insomnia. Fix sleep hygiene, schedule, and stress first.

Does DSIP cause dependency? No chemical dependency has been reported in clinical studies. Some users report a temporary dip in sleep quality when they stop after prolonged daily use, but this is not the same as withdrawal. Cycling prevents this from becoming an issue.

Can I use DSIP for shift work? It has potential. DSIP modulates circadian rhythm signals and promotes deep sleep regardless of time of day. For shift workers, dose 30 to 60 minutes before your target sleep time, regardless of what the clock says. However, no published studies have specifically tested DSIP in shift workers.

Does DSIP make you groggy the next morning? At standard doses, no. At higher doses, some users report grogginess or extended sleepiness into the next day. Start low and adjust.

Can I take it intranasally? DSIP can be administered intranasally, and the rat stroke study used intranasal delivery. However, subcutaneous injection is the more commonly used route in practice because dosing is more precise. If using intranasally, the same reconstitution principles apply as other nasal peptides, using sterile saline instead of bac water.

Side Effects

In Published Research

Clinical studies from the 1980s reported DSIP to have a favorable safety profile. No dependency, tolerance, or significant adverse effects were observed. The withdrawal study of 107 patients noted only headaches in a few patients. No respiratory depression (unlike opioids or benzodiazepines), no rebound insomnia, and no impairment of memory consolidation were reported.

However, long term safety data is limited. These were short duration studies with small sample sizes.

What Users Report

Common (mild):

- Mild headache
- Temporary grogginess if dose is too high
- Slight mood fluctuations
- Vivid dreams (not necessarily negative)

Less common:

- Brief anxiety in rare cases
- Extended sleepiness into the next day at higher doses
- Temporary worsening of sleep quality upon stopping after prolonged daily use

Important notes:

- No dependency or tolerance reported in clinical studies
- No significant hormonal disruption
- Does not suppress REM sleep
- Does not cause next day sedation at standard doses
- Does not cause respiratory depression